

Offline Data Preprocessing Pipeline

Analysis of preprocessing.py

Automated Research Report

This document provides a detailed breakdown of the offline preprocessing pipeline found in `preprocessing/preprocessing.py`. This pipeline standardizes DICOM/NIfTI scans across multiple datasets (BHSD, CQ500, PhysioNet) into a clean, uniform format ready for 3D Segmentation Models.

Phase 1: Spatial & Geometric Standardization

- **Orthonormalize Direction Cosines**

Why: Some CT scanners save images with slightly skewed transformation matrices. This step mathematically corrects the affine matrix to be perfectly orthogonal (90-degree angles). This is absolutely critical for 3D CNNs, otherwise the convolutions distort the physical anatomy.

- **Isotropic Resampling**

Why: CT scans from different hospitals have different voxel spacings (e.g., 1.0mm in X/Y, but 5.0mm in Z). This resamples every patient to a standard physical resolution (e.g., 1x1x1 mm) using Linear interpolation for images and Nearest Neighbor for masks, ensuring a 3x3x3 CNN kernel always represents the exact same physical distance across all patients.

- **Z-Axis Depth Standardization (standardize_depth)**

Why: Scans have a different number of slices (e.g., 30 vs 150). This pads or crops the Z-axis to a fixed depth (e.g., 128 slices). Crucially, if it crops, it crops from the top and bottom, ensuring the centroid of the hemorrhage is perfectly centered in the crop, preventing the ground truth from being cut off.

Phase 2: Intensity & Contrast Normalization

- **CT Windowing (apply_window)**

Why: Raw Hounsfield Units (HU) range from -1000 to +3000, but blood is only in a very narrow band. This applies a 'Brain Window' (or multi-channel window for brain, bone, and subdural) to clamp intensities, drastically boosting the contrast of the hemorrhage against gray/white matter.

- **Histogram Matching**

Why: Different scanner manufacturers (GE, Siemens, Philips) have different intensity profiles. This forces the intensity histogram of every scan to match a 'golden reference' scan, completely eliminating scanner-specific intensity differences.

- **Z-Score / Min-Max Normalization**

Why: Neural networks struggle with large numbers. This normalizes the windowed intensities to either a strict $[0, 1]$ range or a Gaussian distribution (mean=0, std=1), optimizing gradient flow during backpropagation.

Phase 3: Label Cleaning & Quality Control

- **Mask Morphological Cleaning (clean_mask)**

Why: Removes tiny, isolated false-positive pixels in the manual annotations using Connected Component Analysis. It also uses BinaryFillhole to fill any accident inside a hemorrhage annotation, ensuring the CNN learns solid, continuous label structures.

- **Automated Quality Control (QC) Generation**

Why: Automatically generates a 3-panel visualization (middle slice, max-projection slice with red overlay, and binary mask) for every processed patient. This allows researchers to rapidly scan hundreds of patients visually without loading massive NIfTI files into viewers like 3D Slicer.

- **Reproducibility Hashing (build_metadata)**

Why: Embeds the current Git commit, configuration SHA256 hash, and Python/PyTorch versions directly into the JSON metadata of every processed scan. This ensures full traceability for FDA/CE medical device compliance.